MERIT MEDICAL SYSTEMS, INC.

Prepared for the Merit Medical Case Competition
BYU Ballard Center, Business for Good

This case was prepared as a basis for class discussion rather than to illustrate either effective or ineffective handling of a business situation. The protagonist and her internal dialogue are dramatized for teaching purposes; facts and figures are drawn from public sources cited in the endnotes. The supplier segmentation in Exhibit 3 and the strategy comparison in Exhibit 5 are illustrative composites created for classroom discussion and do not represent named, real suppliers or confidential company data.

Introduction

In late February 2026, Alisha Jerauld, Merit Medical's Vice President of Environmental, Social, and Governance (ESG), had two contradictory reports on her desk. The first was a draft press release for Merit's third Newsweek sustainability recognition in a year.[i] The second was an internal rollout report showing that most of Merit's suppliers had not yet completed the sustainability assessment the company had asked them to fill out. The response rates were thinnest among smaller manufacturers in Mexico and China, exactly the regions Merit's own research had flagged as having the least developed rules for tracking emissions and labor practices.[ii] Merit's growing pile of public awards was starting to invite a harder question: did the story match the substance behind it? Alisha had six weeks to bring the Board's Governance and Sustainability Committee a plan that answered both problems at once.

Company Overview

"Merit Medical Systems, Inc. is a leading manufacturer and marketer of proprietary medical devices used in interventional, diagnostic and therapeutic procedures, particularly in cardiology, radiology, oncology, critical care and endoscopy. We strive to be the most customer-focused company in healthcare. Each day we are determined to make a difference by understanding our customers' needs and innovating and delivering a diverse range of products that improve the lives of people and communities throughout the world. We believe that long-term value is created for our customers, employees, shareholders, and communities when we focus outward and are determined to deliver an exceptional customer experience. Merit Medical Systems, Inc. was founded in 1987 by Fred P. Lampropoulos, Kent W. Stanger, Darla Gill and William Padilla. Initially, we focused our operations on injection and insert molding of plastics. Our first product was a specialized control syringe used to inject contrast solution into a patient's arteries for a diagnostic cardiac procedure called an angiogram. Since that time, our products and product lines have expanded substantially, both through internal research and development projects and through strategic

acquisitions." *—Merit Medical Systems, Inc., "Our Company," 2025 Annual Report*

Why Merit Matters in Healthcare

Merit's devices are the tools clinicians use to work inside the body without open surgery. Its portfolio includes steerable guide wires and diagnostic catheters for navigating blood vessels, drainage catheters, embolotherapy products that block blood flow to tumors or deliver chemotherapy to liver cancers, and peritoneal dialysis catheters for home dialysis.[iii] Merit describes these as disposable devices,[iv] so hospitals reorder them continuously. A missing component at one supplier can become a missing device in a procedure room.

That risk is real across the industry. In a survey by the patient-safety nonprofit ECRI, almost half of healthcare professionals said drug, supply, or equipment shortages had delayed patient treatment, and some reported surgeries rescheduled, postponed, or canceled.[v] The FDA has warned that shortages of devices including hemodialysis catheters forced clinicians to delay care for children, and noted that U.S. manufacturers must report supply disruptions only during a declared public health emergency.[vi] Because a new supplier can take 9–12 months to qualify (Exhibit 2), Merit's supplier relationships are part of patient care, not just procurement.

Merit runs this work with roughly 7,100 associates (Exhibit 1), a sales and clinical support team of more than 700,[vii] and manufacturing sites that include its Utah headquarters, Galway (Ireland), Tijuana (Mexico), and Singapore. Merit calls Tijuana one of its primary manufacturing and assembly facilities.[viii]

Merit's Goals and Values

Merit's stated vision is to be the most customer-focused company in healthcare,[ix] and its mission is summed up in three words: Understand, Innovate, and Deliver.[x] In 2024, Merit relaunched its core values as "the Merit Way," known by the acronym HEART:[xi]

Value	How Merit describes it	Why it matters for this case
Health	Prioritizing the health and safety of everyone whose lives Merit touches	Covers workers across the value chain, not only Merit employees
Excellence	High standards of honesty, fairness, and integrity, and the expectation that supply chain partners meet the same standards	Puts supplier conduct inside Merit's own values

Value	How Merit describes it	Why it matters for this case
Agility	Country-by-country risk analysis so devices keep reaching patients even during disruption	Ties supplier resilience directly to patient care
Responsibility	Measuring Merit's environmental footprint year after year	Raises the stakes on unmeasured Scope 3 emissions
Teamwork	"We listen"; building an inclusive culture and meaningful work	Suggests suppliers should be heard, not just surveyed

Merit also has hard financial commitments. In February 2024 it launched its **Continued Growth Initiatives** program, with three targets for the three years ending December 31, 2026:[xii]

- 5%–7% compound annual revenue growth (organic, constant currency)
- A non-GAAP operating margin of 20%–22% in 2026
- At least $400 million in cumulative free cash flow

In 2025, Merit delivered 6.8% organic constant-currency growth, a 20.3% non-GAAP operating margin, and more than $215 million in free cash flow, leaving 2026 as the program's final year.[xiii]
Taken together, these goals set the bar for Alisha's plan. It has to serve hospital customers who increasingly screen suppliers on ESG, live up to a value statement that already promises supplier standards, protect the supply continuity that patients depend on, and fit within a company that has publicly committed to margin and cash discipline.

Merit's Sustainability Program

Merit's Board of Directors oversees ESG issues through a Governance and Sustainability Committee, supported by an internal Corporate Sustainability Council that Alisha chairs.[xiv] In 2024–2025, an outside firm ran a structured materiality assessment, surveying employees, suppliers, and customers on which environmental and social issues mattered most, and identified ten priority topics, including the working conditions of employees across Merit's value chain and how fairly Merit manages its size advantage over smaller suppliers.[xv] Merit has also set a goal of cutting its own direct (Scope 1 and 2) greenhouse gas emissions 40 percent by 2030 versus a 2020 baseline. Progress has been mixed: emissions rose 2 percent in 2025 even as renewable electricity use climbed to 27 percent. Merit has not yet finished measuring Scope 3 emissions — those created by suppliers, shippers, and product disposal, which for most manufacturers dwarf direct emissions — and only set 2024 as its baseline year for that count.[xvi]

Problem 1: Engaging a Global, Unequal Supplier Base

Merit's supplier base is large and concentrated: more than 1,030 direct suppliers across 42 countries, with the top 78 accounting for roughly two-thirds of spend, and only about half of all suppliers based in the continental U.S.[xvii] Historically, Merit selected suppliers almost entirely on quality; environmental and social criteria are newer. To close that gap, Merit began routing suppliers through EcoVadis, a third-party platform that scores how responsibly a business operates.[xviii]

The rollout landed unevenly. Large, often publicly traded suppliers — who already report sustainability data to their own customers — generally finished quickly. Many smaller and mid-sized suppliers, especially precision-parts makers and contract manufacturers in Mexico and China, had no dedicated sustainability staff, no emissions-tracking system, and sometimes no English-language documentation of practices they may already follow. For a supplier like this, an EcoVadis request can look less like partnership and more like unfunded homework — the exact risk Merit's own materiality assessment flagged under "management of supplier relationships."[xix] The problem is compounded by geography: Merit's own climate research already identified Mexico and the China/Singapore region as facing the highest risk on this exact issue, because local rules pushing suppliers toward better reporting remain comparatively underdeveloped there.[xx]

This is not simply a matter of Merit's preference: Merit is itself subject to corporate social responsibility (CSR) laws, including the California Transparency in Supply Chains Act and the UK Modern Slavery Act, that require it to monitor labor standards across its own supply chain — so a supplier's sustainability responsiveness has become a compliance question for Merit, not only a reputational one.[xxi]
Exhibit 3 sketches a way to sort suppliers by leverage (spend concentration, sole-source status) and by readiness (size, existing reporting habits, local regulation). "Anchor Partners" — large, sophisticated, already reporting — sit at one end. Small, single-plant manufacturers in Mexico and China, which carry Merit's biggest data and continuity blind spot, sit at the other. Merit's options range from pushing the same EcoVadis requirement on everyone (fast, but risks relationship strain or shallow answers), to narrowing focus to the top 78 suppliers (efficient, but leaves the highest-risk suppliers untouched for years), to a differentiated approach that offers smaller suppliers real support — and shows them what they personally gain — before holding them to the same bar as Merit's largest partners.

Problem 2: Communicating ESG Performance Without Overreaching

Merit's 2025 awards were real and independently earned — three Newsweek recognitions and a Gallup Exceptional Workplace Award following a record 93 percent employee survey participation rate.[xxii] But some of Merit's own numbers still have gaps: Scope 3 is unmeasured, direct emissions rose 2 percent, and

the workplace injury rate ticked up slightly.[xxiii] Spotlighting the awards while glossing over the harder numbers risks a credibility problem the moment someone looks past the press release.
Regulatory ground is also shifting. Merit's sustainability reporting is modeled on the EU's Corporate Sustainability Reporting Directive (CSRD), but in December 2025 EU lawmakers sharply narrowed which companies CSRD applies to and delayed deadlines for many mid-sized firms by two to three years.[xxiv] Separately, the EU's proposed Green Claims Directive — which would have required proof behind claims like "sustainable" — stalled in 2025, but a related, already-binding law will make vague or offset-based "climate neutral" claims illegal starting September 27, 2026.[xxv] For a company collecting sustainability awards, the message is clear: scrutiny of corporate environmental claims is not going away, even as the specific rules keep moving.

The EU's regulatory reach compounds this problem in a way that goes beyond climate-claims wording. Medical devices sold in the EU already require CE marking under the EU Medical Device Regulation (Regulation (EU) 2017/745, MDR), and Merit's own 2025 Annual Report acknowledges that, given the cost and complexity of that certification, the company will in some cases discontinue or postpone a product's EU introduction rather than pursue approval under the newer rules.[xxvi] That trade-off shows up concretely with battery-powered products: any device that incorporates a battery must now also satisfy the EU's separate Batteries Regulation (Regulation (EU) 2023/1542), which layers its own CE marking, technical documentation, and — starting in 2027 — end-user battery-removability design rules on top of MDR, on a compliance track that runs largely independent of the device's own approval.[xxvii] Faced with that, Merit has chosen in at least one product line to sell only the non-battery version of a device in Europe — even though the battery-enabled version is more advanced and more accurate — simply to avoid stacking two overlapping, still-maturing regulatory regimes onto a single product. The same dynamic shows up elsewhere: Merit's own filings flag active EU consideration of restrictions on PFAS materials and on Ethylene Oxide sterilization, both of which the company currently relies on to manufacture and sterilize its products, alongside a patchwork of country-by-country packaging and extended-producer-responsibility rules.[xxviii] Each new rule brings its own declaration of conformity, documentation, and internal sign-off, so Merit's EU compliance effort looks less like managing a fixed checklist and more like continuously re-configuring for a moving target — and that recurring compliance cost eventually has to be recovered somewhere, typically in product price, which cuts against the same cost discipline that anchors Merit's broader strategy.

At the same time, silence has a cost: hospital systems increasingly screen suppliers on ESG performance, some of Merit's own manufacturing customers are themselves affected by CSRD, and Merit's recruiting data suggests its employee-friendly reputation is becoming a real hiring advantage in competitive labor markets. The task is not whether to tell Merit's story, but how to tell it so it builds trust with investors,

hospital customers, employees, and the suppliers now being asked to open their own books.

The Question Before Alisha

Alisha has six weeks before she presents to the Board's Governance and Sustainability Committee. She needs a supply chain sustainability strategy the Committee can approve and Merit can start piloting this year.

Merit's top goal: build a credible, verifiable picture of sustainability performance across its supply base, one Merit can act on and defend publicly, while strengthening the supplier relationships that keep devices reaching patients.

The stakeholders who matter most to that goal:

1. **Suppliers**, especially the high-power, low-readiness manufacturers in Mexico and China (Exhibit 3)
2. **Hospital and health-system customers**, who increasingly screen suppliers on ESG performance
3. **The Board and investors**, who will weigh any plan against the Continued Growth Initiatives targets
4. **Merit associates**, especially the procurement and quality teams who manage supplier relationships every day

Two common strategies are summarized in Exhibit 5: (A) require every supplier to complete EcoVadis on a fixed deadline, or (B) focus first on the 78 suppliers that hold about two-thirds of spend. Merit is open to either, a combination, or a completely different approach, as long as your proposal makes these five things clear:

1. **Who** Merit engages first, and why, and what waits until later
2. **What** Merit asks of suppliers, and what suppliers get in return
3. **What it costs**, who pays, and how it fits Merit's margin and cash targets
4. **How Merit will know it's working:** metrics, targets, and a timeline
5. **What Merit will say publicly**, to whom, and how it avoids claiming more than its data supports

CENTRAL QUESTION

What supplier sustainability strategy should Merit adopt to build credible, verifiable sustainability data across its global supply base, strengthen the supplier relationships patients depend on, and turn that progress into trust and competitive advantage with customers, investors, and employees, in a way that reflects Merit's values and fits its financial commitments?

Your Strategy Should

1. Serve as a template that works for the suppliers you start with and can scale across Merit's ~1,030 suppliers in 42 countries.
2. Improve the coverage and quality of Merit's supplier sustainability data without disrupting supply continuity or overburdening smaller suppliers (see the TOMS example in Resources).
3. Reflect Merit's vision, mission, and HEART values, and fit within its Continued Growth Initiatives commitments.
4. Answer the following three questions:
    a. Where should Merit focus first, and why? (Consider Exhibits 2 and 3.)
    b. How does your strategy build trust and competitive advantage with the four stakeholder groups above, including what information Merit shares, with whom, and when?
    c. What does your strategy cost, what does it return, and how will Merit measure success?

Teams from every major are encouraged. You may lead with an operations, finance, marketing and communications, strategy, information systems, public policy, global studies, or other lens. Whatever your angle, your proposal must still work within the supply chain realities described in this case.

Deliverables

Each team will submit and present a slide deck. Use public data and clearly labeled assumptions; confidential Merit data is not expected. We suggest the following structure:

Section	Slides	What it should show
Recommendation and rationale	3–4	Your chosen strategy, why it beats the alternatives, and how it advances Merit's goals
Implementation and pilot	2–3	Which suppliers first, rollout steps, owners, timeline, and supplier support
Financials and metrics	2	Costs, expected value, and KPIs with numeric targets
Risks and mitigation	1–2	The biggest risks to suppliers, patients, and Merit, and how you reduce each
Communication plan	1	What Merit shares, with whom, and

Section	Slides	What it should show
		the guardrails against greenwashing
Social impact cycle	1	Your strategy mapped onto the social impact cycle

Evaluation Criteria (in order of importance)

- **Strategic rationale and fit with Merit's goals:** a clear, defended choice that advances Merit's top goal and reflects its vision, mission, HEART values, and financial commitments.
- **Feasibility and scalability:** realistic strategies Merit could start this year and extend across a global supplier base, operationally and financially.
- **Quantitative rigor:** recommendations backed by defensible data analysis, with costs, returns, and measurable targets.
- **Effective social impact:** improves outcomes for suppliers, their workers, and patients while avoiding unintended harm, such as pushing out small suppliers or overstating progress.
- **Operational considerations:** detailed pilot design, process, owners, rollout timeline, and risk mitigation.
- **Creativity and innovation:** novel, compelling ideas that align with Merit's mission.

Prize and Mission Impact

The top three teams will receive cash prizes:

- **1st place:** $3,000
- **2nd place:** $2,000
- **3rd place:** $1,000

Strong proposals will show how a medical device company can raise standards across a global supply chain while strengthening the suppliers, workers, and patients who depend on it.

Resources to Consider

- Merit Medical 2025 Sustainability Report and 2025 Annual Report (Form 10-K), cited throughout the case
- Merit's Vision and Values and At the HEART of Sustainability
- Merit's Continued Growth Initiatives announcement
- EcoVadis (ecovadis.com) for how supplier ratings work
- TOMS Shoes' example of unintended negative impacts in social impact: Wharton article

- FTC Green Guides (U.S. guidance on environmental marketing claims)
- Social impact cycle
- Exhibits 1–5

EXHIBITS

Exhibit 1

Merit Medical: Key Figures at a Glance (2025, 2024, 2020 Baseline)

Metric	2025	2024	2020 (Baseline)
Net Sales ($K)	1,515,906	1,356,514	963,875
Net Income ($K)	128,489	120,357	(9,843)
Total GHG Emissions (MT CO2e, Scopes 1+2)	30,875	22,688	31,452
% Energy from Renewable Sources	27%	18%	—
Merit Associates (avg.)	7,110	6,885	6,004
Total Recordable Incident Rate (TRIR)	1.0	0.9	1.1

Source: Merit Medical Systems, Inc., 2025 Sustainability Report, "Key Figures at a Glance," p. 7.

Exhibit 2

Merit's Supply Chain by the Numbers (2025)

Metric	Value
Direct suppliers	~1,030
Countries represented	42
Share of annual spend held by top 78 suppliers	~65%
Suppliers located in continental U.S.	49%
Purchases from diversity-designated suppliers	11%
Supplier audits conducted in 2025	29
Typical FDA supplier (re)qualification timeline	9–12 months

Source: Merit Medical Systems, Inc., 2025 Sustainability Report, "Supply Chain Management," p. 35, and "Quality Assurance,"

p. 40.

Exhibit 3

Illustrative Supplier Power–Readiness Segmentation (Composite, for Case Discussion Only)

Note: The categories below are illustrative composites created for classroom discussion. They do not represent actual, named Merit suppliers or confidential company data.

	Low Sustainability Readiness	Moderate Readiness	High Readiness
High Power (sole-source / high spend)	Critical single-plant component makers, Mexico/China — high switching cost, limited capacity	Mid-size specialty manufacturers with partial reporting systems	"Anchor Partners": large, often public, multinational suppliers already reporting on their own
Low Power (fragmented spend, substitutable)	Small commodity-input suppliers with minimal capacity	Regional distributors and packaging vendors building basic reporting	Smaller suppliers already certified via other customers' requirements

Framework adapted from standard stakeholder power–interest mapping, applied here to supplier sustainability engagement.

Exhibit 4

2025–2026 Recognition and Reporting Frameworks Referenced in This Case

Item	What It Is
Newsweek: America's Most Responsible Companies 2026	Annual ranking based on social responsibility performance and stakeholder commitments
Newsweek: World's Greenest Companies 2025	Annual ranking based on environmental footprint reduction
2026 Gallup Exceptional Workplace Award	Recognizes strong employee engagement, based on survey data
GRI (Global Reporting Initiative)	Voluntary, widely used template for what companies disclose about social and environmental impact
SASB Standards	Industry-specific disclosure standards, including ones written for medical device makers
TCFD	Framework for explaining how climate risk could affect

Item	What It Is
	a company financially
EU CSRD	EU law requiring many large companies to publish audited sustainability reports; scope and timelines were narrowed in a Dec. 2025–Feb. 2026 simplification package
EU Green Claims Directive / ECGT	Proposed EU rule requiring proof behind green marketing claims (stalled in 2025); a related, already-binding EU law bans vague or offset-based "climate neutral" claims starting Sept. 27, 2026
EU Medical Device Regulation (MDR)	Regulation (EU) 2017/745; requires CE marking for medical devices sold in the EU; replaced the earlier Medical Device Directive as of May 2021
EU Batteries Regulation	Regulation (EU) 2023/1542; imposes a separate CE marking and documentation process, plus (from 2027) end-user battery-removability requirements, on devices that contain batteries

Exhibit 5

Two Common Supplier Engagement Strategies Under Consideration

These are starting points drawn from the case, not the expected answer. Teams may adopt, combine, or replace them.

	Strategy A: Uniform mandate	Strategy B: Top-78 focus
What it is	Every direct supplier completes an EcoVadis assessment by a fixed deadline	Engage the 78 suppliers holding ~65% of spend first; others later
Supplier coverage	~1,030 suppliers in 42 countries	78 suppliers, mostly large "Anchor Partners"
Speed to data	Fast on paper; real response depends on supplier capacity	Fast and high-quality for the suppliers covered
Burden on suppliers	Heaviest on small suppliers with no sustainability staff	Light; most top suppliers already report

	Strategy A: Uniform mandate	Strategy B: Top-78 focus
What it leaves exposed	Shallow or incomplete answers; strained relationships with hard-to-replace suppliers	The small, single-plant suppliers in Mexico and China that carry Merit's biggest data and continuity blind spot
Fit with Merit's goals	Strong on Excellence (one standard for all); weaker on Teamwork and Agility	Strong on cost discipline; weaker on Health and Responsibility across the full value chain

Source: Case text and Exhibits 2–3; Merit Medical Systems, Inc., 2025 Sustainability Report, p. 35.

Endnotes

i. Merit Medical Systems, Inc., *2025 Sustainability Report* (South Jordan, UT: Merit Medical, 2026), p. 42.
ii. Merit Medical, *2025 Sustainability Report*, pp. 20–22, 35.
iii. Merit Medical Systems, Inc., *2025 Annual Report (Form 10-K)* (South Jordan, UT: Merit Medical, 2026), "Business."
iv. Merit Medical, "An Introduction to Merit Medical," merit.com, accessed October 2026.
v. MedTech Dive, "Supply Disruptions Delaying Surgeries, Treatments: ECRI" (report on the ECRI and Institute for Safe Medication Practices survey).
vi. Becker's Hospital Review, "Device Supply Gaps Put Pediatric Patients at Risk: FDA," January 2025.
vii. Merit Medical Systems, Inc., Form 8-K, Exhibit 99.3, February 28, 2024, "About Merit."
viii. Merit Medical, "The Making of Merit Medical—A Timeline Celebrating 35 Years," merit.com.
ix. Merit Medical, "Vision & Values," merit.com, accessed October 2026.
x. Merit Medical, "Building Better Healthcare: The Merit Medical Story," merit.com.
xi. Merit Medical, "At the HEART of Sustainability: A Look at Our 2024 Progress," July 24, 2025.
xii. Merit Medical Systems, Inc., "Merit Medical Introduces 'Continued Growth Initiatives' Program and Financial Targets for Three-Year Period Ending December 31, 2026," Form 8-K, Exhibit 99.3, February 28, 2024; targets reiterated in "Merit Medical Systems Q2 Earnings Call Highlights," July 2026.
xiii. Merit Medical Systems, Inc., "Merit Medical Reports Fourth Quarter and Full Year 2025 Results and Issues Fiscal Year 2026 Guidance," Form 8-K, Exhibit 99.1, February 24, 2026.
xiv. Merit Medical, *2025 Sustainability Report*, pp. 13–14.
xv. Merit Medical, *2025 Sustainability Report*, pp. 9–11.
xvi. Merit Medical, *2025 Sustainability Report*, pp. 16–17.
xvii. Merit Medical, *2025 Sustainability Report*, p. 35.
xviii. Merit Medical, *2025 Sustainability Report*, p. 35.
xix. Merit Medical, *2025 Sustainability Report*, pp. 9–11.
xx. Merit Medical, *2025 Sustainability Report*, p. 22.
xxi. Merit Medical, *2025 Annual Report (Form 10-K)*, "Regulation," p. 11 (California Transparency in Supply Chains Act and UK Modern Slavery Act monitoring obligations).
xxii. Merit Medical, *2025 Sustainability Report*, pp. 31, 42.
xxiii. Merit Medical, *2025 Sustainability Report*, pp. 17, 29.
xxiv. European Commission, "Corporate Sustainability Reporting," Finance, accessed 2026, https://finance.ec.europa.eu; PwC,

"'Omnibus' Directive Finalised," February 2026, https://viewpoint.pwc.com.

xxv. European Parliament, "'Green Claims' Directive: Protecting Consumers from Greenwashing," Think Tank Briefing, 2023, https://www.europarl.europa.eu; on the Commission's 2025 move to withdraw the proposal and the related, already-binding Empowering Consumers for the Green Transition Directive (effective September 27, 2026), see Inside Energy & Environment, "The European Commission's New Green Claims Guidance," December 2025, https://www.insideenergyandenvironment.com.

xxvi. Merit Medical, *2025 Annual Report (Form 10-K)*, "Regulation," p. 12 (EU Medical Device Regulation (EU) 2017/745 and its effect on MDR certification costs and EU product decisions).

xxvii. Regulation (EU) 2023/1542 of the European Parliament and of the Council of 12 July 2023 on Batteries and Waste Batteries, https://eur-lex.europa.eu; Emergo by UL, "EU Deadline Approaching for Portable Batteries in Medical Devices and IVDs," July 16, 2024, https://www.emergobyul.com.

xxviii. Merit Medical, *2025 Annual Report (Form 10-K)*, "Environmental, Health and Safety and Corporate Social Responsibility Risks," p. 30 (PFAS and Ethylene Oxide sterilization restrictions under EU consideration).